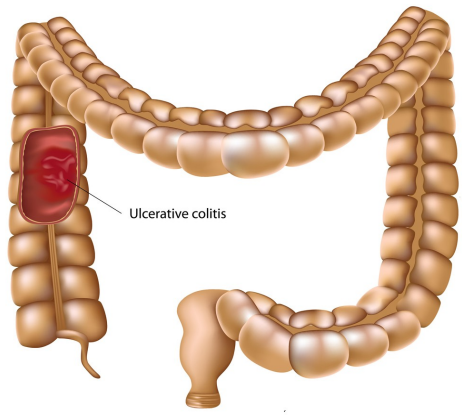
Ulcerative colitis is a chronic, inflammatory disease primarily affecting the mucosa of the rectum and distal colon. Characterized by exacerbations and remissions, it affects female patients more often than male patients, most often appearing between the ages of 15 and 20 years of age. It has a smaller, second peak between 55 and 60 years of age. About 40 to 50% of ulcerative colitis patients have the disease limited to the rectum and rectosigmoid, 30 to 40% have disease beyond the sigmoid yet not affecting the whole colon, and 20% will have disease of the entire colon. Although there are psychological factors that have been implicated in its cause, the disease is mainly speculated to have a genetic origin.

Typically, ulcerative colitis is a chronic disease, with symptoms of a low-grade nature. These symptoms include bloody stools, pain, malaise, and diarrhea. A smaller percentage (around 15%) of patients will develop an acute ulcerative colitis characterized by profuse rectal hemorrhage, severe abdominal pain, and high fever. Other symptoms associated with the disease include marked weakness, weight loss, and nausea and vomiting. In patients with active disease, there can be an increase in acute-phase reactants (orosomucoid levels, C-reactive protein), erythrocyte sedimentation rate, platelet count, and a decrease in hemoglobin. In patients who are severely ill, the serum albumin level will rapidly decline, and there may be leukocytosis.



Toxic megacolon is associated with an acute onset of fulminating ulcerative colitis, resulting in severe colonic distention that leads to shock. Patients with toxic megacolon often have bacterial overgrowth because the distended bowel lumen provides an excellent environment for their growth. Bacterial-produced toxins can be systemically released due to this condition, coupled with perforation and erosive intestinal inflammation. The most dangerous local complication is bowel perforation. It carries an approximate mortality rate of 15%, with the physical signs of peritonitis not being obvious, especially in the patients being treated with glucocorticoids.

Patients affected by ulcerative colitis are at an increased risk for carcinoma of the colon. Large-joint arthritis can be seen in patients with clinically active disease. There can be concomitant liver disease, as well as ankylosing spondylitis, iritis, and erythema nodosum.

Initial treatment for ulcerative colitis is typically medical. Surgical intervention is reserved for patients whose complications are intractable. Treatment regimens include antidiarrheal agents and sulfasalazine preparations, as well as glucocorticoids. Antibiotics are not useful in the treatment of ulcerative colitis.

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